

Al Noor General Hospital

Patient Care and Safety Policy Manual

Version 3.0 | Effective January 2026 | Sample document for testing

1. Purpose, Scope and Patient Rights

1.1 Purpose. This manual sets out how Al Noor General Hospital delivers safe, respectful and consistent care to every patient. It applies to all clinical and non-clinical staff, contractors, students and volunteers working in any ward, clinic, emergency department or diagnostic area of the hospital.

1.2 Patient rights. Every patient has the right to respectful care regardless of nationality, religion, gender, age or ability. Patients have the right to know the name and role of the staff caring for them, to receive clear information about their diagnosis and treatment, and to take part in decisions about their care.

1.3 Privacy and dignity. Patients must be examined behind a closed door or curtain. Intimate examinations require a chaperone of the patient's choice where possible. Patient information is confidential and shared only on a need-to-know basis.

1.4 Responsibilities of patients. Patients are asked to give accurate information about their health, follow the agreed treatment plan, treat staff and other patients with courtesy, and follow the hospital's visiting and safety rules.

1.5 Related documents. This manual should be read together with the Infection Control and Occupational Health Policy and the Staff Handbook. Where a topic is covered in more than one document, the documents are written to agree with each other, and the same rule applies everywhere.

1.6 Review. The Quality Department reviews this manual every year and after any serious patient safety event. Staff receive a summary of any change through their department head.

2. Visitor Policy

2.1 Visiting hours. Visiting hours for all general wards are 4:00 PM to 8:00 PM, every day including weekends and public holidays. Visitors must register at the main reception and wear a visitor badge for the whole visit.

2.2 Number of visitors. A maximum of 2 visitors are allowed at a patient's bedside at the same time. Additional family members may wait in the ground-floor lounge.

2.3 Children. Children under 12 years are not permitted in general wards unless the ward nurse in charge gives written approval.

2.4 Intensive Care Unit. ICU visits are limited to 15 minutes per visit and 1 visitor at a time. Every ICU visit must be approved by the nurse in charge, who may refuse or shorten a visit for clinical reasons.

2.5 Health screening of visitors. Visitors with fever, cough, diarrhoea or any other infectious symptom must not enter patient areas. Reception staff may ask visitors screening questions and may refuse entry.

2.6 Items. Flowers, uncooked food and outside medication are not allowed in patient rooms. Visitors should check with the nurse before bringing any food because many patients follow special diets. Mobile phones must be on silent inside the wards.

2.7 Behaviour. Visitors must follow staff instructions at all times. Staff may ask a visitor to leave if the visitor disturbs other patients or refuses to follow the rules. Staff must call hospital security on extension 100 if a visitor is aggressive, and must not argue with the visitor.

3. Admission and Patient Identification

3.1 Documents at admission. Patients must bring a valid Emirates ID or passport, their insurance card and any previous medical reports. Emergency patients are admitted first, and their documents may be submitted within 24 hours.

3.2 Two-identifier rule. Staff must confirm a patient's identity using two identifiers, the full name and the date of birth, before giving medication, taking blood samples, performing a procedure or transporting the patient. The room number is never used as an identifier.

3.3 Identification wristband. Every admitted patient receives a wristband showing the full name, date of birth and medical record number. Patients with allergies wear a red allergy wristband. A damaged or missing wristband must be replaced immediately.

3.4 Initial assessment. A registered nurse completes the nursing admission assessment, including allergies, current medication, pain, falls risk and skin condition, on arrival to the ward. The treating doctor completes the medical assessment as soon as possible after admission.

3.5 Personal belongings. Patients are encouraged to leave valuables at home. Valuables handed to the nursing staff are listed, signed for by two staff members and stored in the ward safe. The hospital is not responsible for items kept at the bedside.

3.6 Orientation. New patients are shown how to use the nurse call button, the bed controls and the bathroom alarm, and are told the visiting hours and the meal times of the ward.

4. Consent and Communication

4.1 Informed consent. Written informed consent is required before surgery, anaesthesia, blood transfusion and any invasive procedure. The doctor performing the procedure must explain the purpose, benefits, risks and alternatives in language the patient understands.

4.2 Capacity. A patient is presumed to have capacity to decide unless assessed otherwise. If a patient lacks capacity, consent is sought from the legally authorised representative. In an emergency where delay would risk life, treatment may proceed in the patient's best interest and must be documented.

4.3 Interpreters. The hospital provides a free interpreter on request for patients who do not speak English or Arabic. Family members should not be used as interpreters for consent discussions unless the patient insists and a staff member records this.

4.4 Refusal of treatment. A patient with capacity may refuse treatment. The doctor must explain the possible consequences, document the discussion and ask the patient to sign a refusal form.

4.5 Breaking bad news. Serious news is shared by the treating doctor in a private room with enough time for questions. Patients may ask for a family member or friend to be present.

4.6 Communication at the bedside. Staff introduce themselves by name and role at each shift change. A whiteboard at the bed shows the nurse's name, the doctor's name and the plan for the day.

5. Clinical Monitoring and Pain Management

5.1 Vital signs. Vital signs, including temperature, pulse, blood pressure, breathing rate and oxygen saturation, are recorded at least every 6 hours for patients in general wards. Unstable patients are monitored more often as ordered by the doctor.

5.2 Early warning score. Every set of vital signs is converted into an early warning score. A high score must be reported to the nurse in charge immediately, and the nurse in charge decides whether to call the Rapid Response Team.

5.3 Pain assessment. Pain is assessed using a 0 to 10 scale, where 0 is no pain and 10 is the worst pain imaginable. Pain is assessed at least every 4 hours and again 30 minutes after any pain medication is given.

5.4 Pain management. Staff offer non-drug measures, such as repositioning and cold or warm packs, together with prescribed medication. Persistent pain that does not improve must be reported to the doctor.

5.5 Fluid and nutrition. A dietitian screens every adult patient for malnutrition risk within 24 hours of admission. Fluid intake and output are recorded for patients on strict fluid monitoring.

5.6 Documentation. All observations and care are documented at the time they are done, signed and timed. Late entries are clearly marked as late entries.

6. Medication Safety

6.1 Five rights. Before giving any medication, the nurse checks the five rights: the right patient, the right medication, the right dose, the right route and the right time.

6.2 Allergy check. The patient's allergy status must be checked and recorded before the first dose of any new medication. Patients with a red allergy wristband are checked twice.

6.3 High-alert medications. Insulin, heparin, concentrated electrolytes, chemotherapy and opioids are high-alert medications. Two nurses must independently check the dose before these medications are given.

6.4 Prescriptions. Prescriptions must be written clearly with the full generic name, dose, route and frequency. Verbal orders are accepted only in emergencies and must be written down, read back and signed by the doctor within 24 hours.

6.5 Storage. Medication is stored in locked cupboards or locked medication rooms. Controlled drugs are kept in a double-locked cupboard, and the controlled drug register is checked and signed by two nurses at every shift change.

6.6 Medication errors. Any medication error or near-miss must be reported in the incident reporting system. The nurse must also inform the doctor immediately so that the patient can be treated if needed.

6.7 Patient's own medication. Medication brought from home is checked by the pharmacist and may be used only if the doctor approves. Otherwise it is stored safely and returned at discharge.

7. Falls Prevention and Pressure Injury

7.1 Falls risk assessment. A falls risk assessment is completed within 4 hours of admission, repeated every 24 hours, and repeated again after any fall or change in the patient's condition.

7.2 Preventive measures. For patients at risk of falling, staff keep the bed in the lowest position, keep the call bell and personal items within reach, ensure the floor is dry and uncluttered, and provide non-slip footwear. High-risk patients wear a yellow falls-risk wristband.

7.3 After a fall. The patient must not be moved until a nurse has assessed for injury. The doctor is informed immediately, observations are recorded, and the fall is reported in the incident reporting system.

7.4 Pressure injury risk. A skin assessment is completed on admission and at least once every day. Patients who cannot move themselves must be repositioned at least every 2 hours, and the time and position are documented.

7.5 Skin care. Staff keep the skin clean and dry, use pressure-relieving mattresses for high-risk patients, and refer any new skin damage to the wound care nurse the same day.

7.6 Mobility. Patients are encouraged to walk or sit out of bed as soon as the doctor allows, because early movement reduces both falls risk and pressure injury.

8. Emergency Response

8.1 Code Blue. A cardiac or respiratory arrest is a Code Blue. Any staff member who finds a patient who is not breathing must call extension 555 and start cardiopulmonary resuscitation (CPR) until the Code Blue team arrives.

8.2 Rapid Response Team. The Rapid Response Team (RRT) is called when a patient's condition is getting worse but has not yet reached arrest. The team arrives within 5 minutes of the call, and the nurse in charge stays with the patient until it arrives.

8.3 Emergency equipment. Each ward has a resuscitation trolley with a defibrillator. The trolley is checked every day and after every use, and the check is signed on the trolley checklist.

8.4 Fire. In case of fire, staff follow the RACE steps: Rescue, Alarm, Contain and Extinguish or Evacuate. The assembly point for evacuated patients and staff is Car Park A.

8.5 Security emergencies. Threats, violence or missing patients are reported to hospital security on extension 100. A missing patient alert is raised if a patient cannot be found within 15 minutes of being noticed missing.

8.6 Mass casualty. The hospital disaster plan is activated by the Medical Director or the duty administrator. Staff report to their assigned areas and follow the instructions of the incident commander.

9. Discharge Planning and Discharge

9.1 Discharge time. The standard discharge time is 12:00 noon. Patients who stay in the room after 12:00 noon may be charged for an additional day.

9.2 Planning. Discharge planning starts at admission. The ward team discusses the expected discharge date with the patient and family and arranges home care, equipment or follow-up appointments where needed.

9.3 Discharge process. The discharge process starts after the treating doctor signs the discharge summary and the billing department clears the final bill. The pharmacy then prepares the discharge medication.

9.4 Patient education. Before the patient leaves, the nurse explains the medication, warning signs that need urgent attention, diet and activity advice, and the date of the follow-up appointment. The nurse uses teach-back to check understanding.

9.5 Discharge summary. The patient receives a copy of the discharge summary that lists the diagnosis, treatment given, medication and follow-up plan.

9.6 Discharge against advice. A patient with capacity may choose to leave against medical advice. The doctor explains the risks, and the patient signs the discharge against medical advice form. Treatment is not denied in the future because of this decision.

9.7 Transport. Patients and families should arrange transport in advance. The ward can help to arrange hospital transport for patients who need an ambulance.

10. Complaints, Appointments, Billing and Medical Records

10.1 Complaints. Patients and families can give feedback or complaints to any staff member, to the Patient Relations office on the ground floor, or by email to feedback@alnoor-hospital.example. The hospital replies within 3 working days.

10.2 Appointments. Appointments can be booked by calling 800-NOOR, using the hospital app, or visiting the reception desk. Patients should arrive 15 minutes early with their Emirates ID and insurance card.

10.3 Cancellations. Appointments must be cancelled at least 24 hours before the appointment time. A late cancellation or no-show may be charged a fee of AED 100.

10.4 Billing. The hospital accepts cash, credit and debit cards, and direct insurance billing for approved insurers. Insured patients may still need to pay a co-payment at the billing desk.

10.5 Medical records. Medical records are kept for 25 years. A patient may request a copy of their medical records in writing at the Medical Records office with valid identification. The hospital responds within 5 working days, and the first copy is free.

10.6 Who may see records. Records may be shared only with the patient, the patient's authorised representative, the treating team, or as required by law. Staff must not look at the record of a patient they are not caring for.